

Texas Veterinary Specialists — Internal Medicine

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SPECIALIST CONSULTATION NOTE — INTERNAL MEDICINE

Note Type:	CONSULTATION	Date of Visit:	2024-11-02
Patient:	Charlie	Owner:	Marcus Williams
Species:	Canine	Breed:	Labrador Mix
DOB:	2015-03-22	Weight:	32.1 kg

REFERRAL REASON

Referred by Dr. Sarah Okonkwo (Greenfield Animal Hospital) for evaluation of persistently elevated hepatic enzymes (ALT 3x upper reference limit, ALP 5x upper reference limit) identified on routine bloodwork 3 weeks ago, with repeat panel confirming persistence. Charlie is a 9-year-old male neutered Labrador Mix with a history of seasonal allergies and hypothyroidism (well-controlled on levothyroxine 0.5 mg PO SID for 2 years).

HISTORY

Owner reports mild decrease in appetite over 6 weeks. No vomiting or diarrhoea. Slightly increased water intake and urination noted over the past month (owner not certain). Exercise tolerance decreased — tires on walks that were previously tolerated. No known toxin or drug exposure beyond current medications. No recent dietary changes. No travel outside of Central Texas.

PHYSICAL EXAMINATION

Parameter	Value	Reference Range	Status
Temperature	38.6 C	37.8–39.2 C	Normal
Heart rate	84 bpm	60–100 bpm	Normal
Respiratory rate	20 bpm	10–30 bpm	Normal
Weight	32.1 kg	—	Stable
BCS	5/9	—	Ideal

General: BAR. Mild muscle wasting dorsal epaxial muscles. Coat dull. Abdomen: Soft; mild hepatomegaly palpated. No ascites. Lymph nodes: No peripheral lymphadenopathy. Eyes: Mild nuclear sclerosis bilaterally (age-appropriate). Mucous membranes: Pink, moist. CRT < 2 seconds. No icterus.

DIAGNOSTIC FINDINGS

Abdominal ultrasound (performed today): Hepatomegaly with diffusely hyperechoic hepatic parenchyma — pattern most consistent with vacuolar hepatopathy or hepatic lipidosis. No focal masses or biliary obstruction identified. Gallbladder: mild sludge, wall normal. Spleen: mildly enlarged, homogeneous. Adrenal glands: left adrenal gland mildly enlarged (0.9 cm width) — borderline; right within normal limits. Fine needle aspirates: liver (2 sites) and spleen submitted to external cytology lab (results pending 3–5 business days). In-house chemistry repeat (today): ALT 312 U/L (ref < 107), ALP 842 U/L (ref < 147), GGT 28 U/L (ref < 11), albumin 2.6 g/dL (ref 2.7–3.8, mildly low), total bilirubin 0.4 mg/dL (normal), glucose 98 mg/dL (normal), BUN/Creatinine within normal limits. Urinalysis: USG 1.018, no glucose, no bilirubin, trace protein.

ASSESSMENT

1. Vacuolar hepatopathy (primary differential) — hepatomegaly with hyperechoic parenchyma and elevated ALP/ALT/GGT; mildly low albumin. 2. Suspected hyperadrenocorticism (Cushing syndrome) — borderline adrenal enlargement, elevated ALP, polyuria/polydipsia history, muscle wasting; cannot confirm without further endocrine testing. Rule-out prior to concluding primary hepatopathy. 3. Hypothyroidism — known, well-controlled on levothyroxine (pre-existing). 4. Hepatic neoplasia — less likely given diffuse pattern and age; awaiting cytology to further characterise.

PLAN

- Await hepatic and splenic cytology results (external lab, 3–5 days). - Low-dose dexamethasone suppression test (LDDS) to investigate hyperadrenocorticism — to be performed at referring clinic within 7 days. - Commence SAMe (Denosyl) 400 mg PO SID and Vitamin E 400 IU PO SID as hepatoprotectants. - Continue levothyroxine at current dose; recheck thyroid panel in 3 months. - Recheck with Dr. Osei in 4 weeks with LDDS results and cytology report. - Owner counselled: prognosis depends on underlying aetiology; Cushing-related hepatopathy typically responds to Cushing treatment.

EXPECTED CODING OUTPUT (test validation reference)

Expected Code	System	Category	Confidence Tier
K76.0	ICD-10-CM	DIAGNOSIS	HIGH
E24.9	ICD-10-CM	DIAGNOSIS	MEDIUM
E03.9	ICD-10-CM	DIAGNOSIS	HIGH
C22.9	ICD-10-CM	DIAGNOSIS	LOW
R63.0	ICD-10-CM	SYMPTOM	HIGH
R35.0	ICD-10-CM	SYMPTOM	MEDIUM

Test assertion: *requires_review* = true (LOW confidence C22.9 hepatic neoplasia) | Cushing coded as MEDIUM (suspected, not confirmed) | *hypothyroidism_is_pre_existing* = true | *hepatic neoplasia confidence_tier* = LOW | *overall_confidence* = LOW